

Environmental
E-06 Hazardous Material Exposure BLS

Revised 3/1/2000

PRIORITIES:

- Safety of rescuers: Do NOT add to the number of victims.
- Park responding units upwind and/or uphill;
- Do NOT walk or drive through spilled materials;
- Do NOT take unnecessary risk that may lead to contamination.
- DECONTAMINATION (a fire department responsibility) takes priority over medical interventions and transport;
- ABCs
- Treat patients symptomatically
- Assure an advanced life support response.

General Procedures (Initiate procedures AFTER decontamination)

1. Evaluate and ensure patient's airway is patent;
2. OXYGEN THERAPY – Begin oxygen 10 liters/minute with non-rebreathing mask and/or bag-valve mask ventilation assistance, as indicated;
3. If indicated, continue flushing contaminated skin with water.

Environmental E-06 Hazardous Materials Exposure

Priorities

- Safety of all responders is priority. Do not enter contaminated areas. Only enter areas that have been deemed safe by HazMat personnel.
- **Decontamination of the exposed patient, personnel, or essential equipment takes priority over any medical intervention and/or transport.**
- Obtain as much information on the hazardous material as possible.
- Make base and/or receiving hospital contact as soon as possible so the receiving hospital can prepare to accept an exposed patient.

General Hazardous Materials Treatment

Decontamination of exposed patients,
personnel, or essential equipment

Stabilize airway using the appropriate adjuncts. Be prepared to use an advanced airway.

Oxygen – Titrate to SpO₂ >95% unless stated in specific treatments

Cardiac monitor

IV/IO Access

NS bolus 500mL for SBP <90mmHg
May repeat once

For severe SOB consider
CPAP at 10mmH₂O

For wheezing

Albuterol 5mg via HHN/Nebulizer Mask/CPAP/BVM
May repeat albuterol only once for continued wheezing

Consider

Ipratropium bromide 0.5mg
via HHN/Nebulizer Mask/CPAP/BVM

May repeat albuterol only once for continued wheezing

Treat specific hazardous
materials per guideline on
subsequent pages

DISRUPTED COMMUNICATIONS

In the event of a “disrupted communications” situation, Solano County Paramedics may not utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.

Environmental E-06 Hazardous Materials Exposure

Irritant gases
(acids, ammonia, chlorine)

Treatment per
general hazmat
treatment

Do not induce vomiting
If evidence of an oral injury,
do not use a King Tube.

Smoke inhalation and
carbon monoxide

Treatment per
general hazmat
treatment

For continued hypotension after NS bolus
Epinephrine drip 1:1,000 in 1,000mL NS
Titrate to SBP >90mmHg

Arsine and
phosphine gas

Treatment per
general hazmat
treatment

Base Hospital Physician Consult
Sodium Bicarbonate 50mEq IV/IO
For alkalization of urine

Hydrogen sulfide/
sulfides/mercaptans

Treatment per
general hazmat
treatment

Irrigate eyes if exposed

Pesticides
Carbamates/Organophosphates

Treatment per
general hazmat
treatment

Atropine 0.5mg-2mg IV/IO
May repeat doses of 2mg-4mg IV/IO q3-10min
No max dose

If actively seizing
Midazolam 10mg IM/IN OR
Midazolam 4mg IV/IO
May repeat once if seizure persists or returns
The preferred route for Midazolam is IM/IN

Contact base for additional drug doses

Cyanide

Treatment per
general hazmat
treatment

Cyanide antidote kit may be
administered if available on
scene by site personnel and
directed to administer by Base
Hospital Physician order only.

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Hydrofluoric acid

Treatment per
general hazmat
treatment

Do not induce vomiting.
Dilute internal ingestion with PO water in
patients that are awake and have a gag
reflex.
If ingested, do not use a King Tube for
airway management.

**For high concentration (10-20%)
exposure to >3-5% BSA (face, neck, or
back), or cardiac arrest, or hypocalcemic
muscle spasms**

Calcium chloride 10% 10mL slow IV/IO
No faster than 1mL/min

For isolated extremity exposure with <3-5% BSA

Calcium Chloride 10%

Pour contents of one ampule into a sterile glove or
container and immerse affected area into solution.
If calcium gluconate gel or other calcium binding
agent has been applied, no further treatment is
necessary

Petroleum distillates and halogenated hydrocarbon solvents

Treatment per
general hazmat
treatment

Irrigate eye injuries.
Do not induce vomiting.
Avoid administration of epinephrine and
bronchodilators.

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